

Name : Saroj Agarwal	Age/Gender : 63 y / F	Collected : 13/04/2026 11:05
Patient ID : 0010663186	Visit Type : IP	Received : 13/04/2026 12:29
Referred By : Dr. Darshit Shah	DOB : 27/08/1962	Reported : 13/04/2026 14:28
Accession : 2600080971	Location : 1J7C`1J7CR03`1JB703`1ONCOMED	

DEPARTMENT OF LABORATORY MEDICINE - BIOCHEMISTRY

	Test Name	Unit	Reference Range	Low	Normal	High
	Liver Profile					
	Bilirubin Total + Direct					
	Total Bilirubin (Serum, Diazo)	mg/dL	<= 1.2		0.23	
	Direct Bilirubin (Serum, Diazo)	mg/dL	<= 0.3		0.16	
	Indirect Bilirubin (Serum, Calculated)	mg/dL	<= 1.2		0.07	
	Alanine Aminotransferase [ALT/PT] (Serum, IFCC-Enzymatic with P5P)	U/L	<= 35			39.20
	Aspartate Aminotransferase [AST/OT] (Serum, IFCC-Enzymatic with P5P)	U/L	<= 35		33.60	
	Gamma Glutamyltransferase [GGT] (Serum, Enzymatic IFCC)	U/L	< 40			47.90
	Alkaline Phosphatase [ALP] (Serum, Enzymatic IFCC)	U/L	35-104		77.00	
	Proteins					
	Total Protein (Serum, Colorimetric - biuret)	g/dL	6.4-8.3	4.63		

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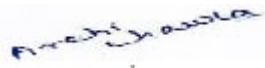
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	Test Name	Unit	Reference Range	Low	Normal	High
	Albumin (Serum, Colorimetric - BCG)	g/dL	3.5-5.2	2.73		
	Globulin (Serum, Calculated)	g/dL	2-3.5	1.90		
	A / G ratio (Serum, Calculated)		1.0-2.5		1.44	

Total Bilirubin:

- Diseases or conditions which, through hemolytic processes, produce bilirubin faster than the liver can metabolize it, cause the levels of unconjugated (indirect) bilirubin to increase in the circulation.
- Liver immaturity and several other diseases in which the bilirubin conjugation mechanism is impaired cause similar elevations of circulating unconjugated bilirubin.
- Bile duct obstruction or damage to hepatocellular structure causes increases in the levels of both conjugated (direct) and unconjugated (indirect) bilirubin in the circulation.

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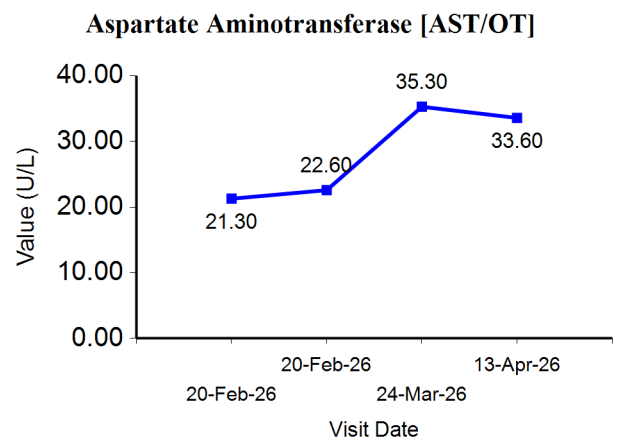
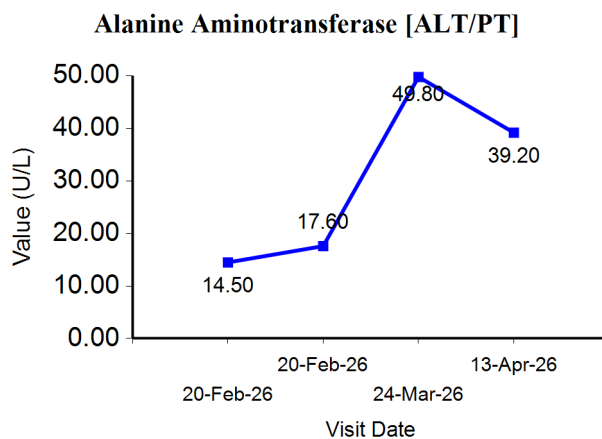


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Trend Analysis & Graph



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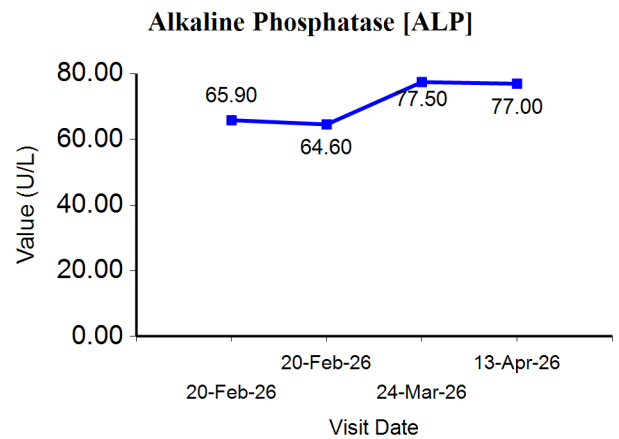
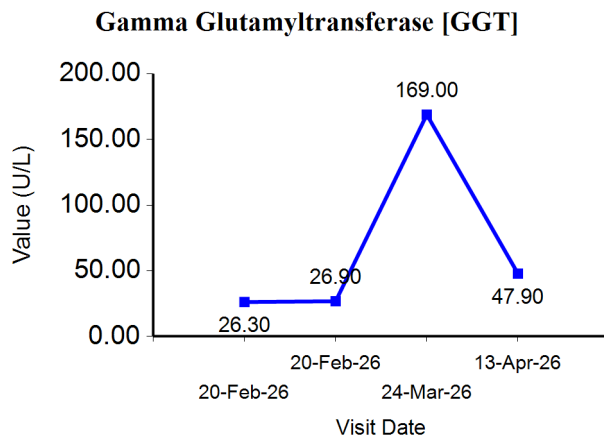
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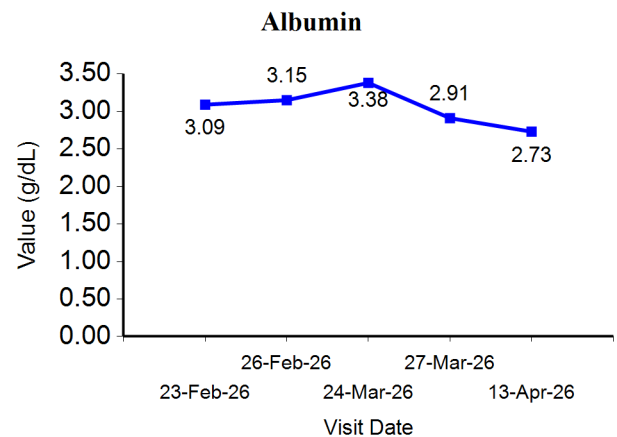
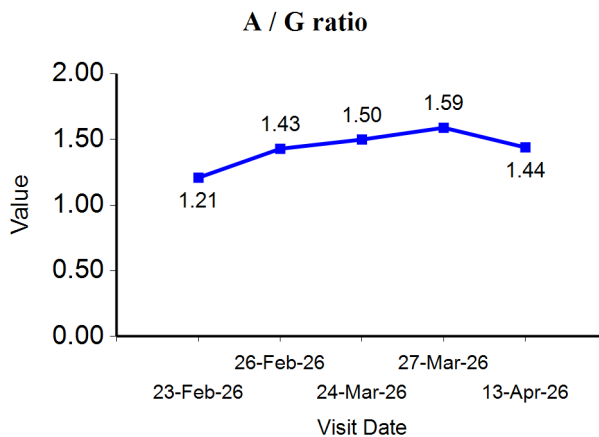


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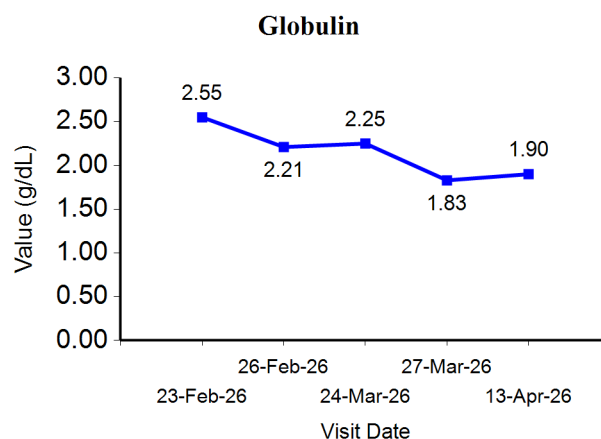
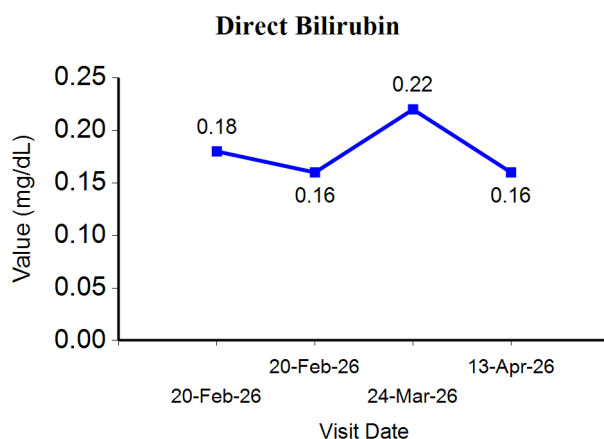
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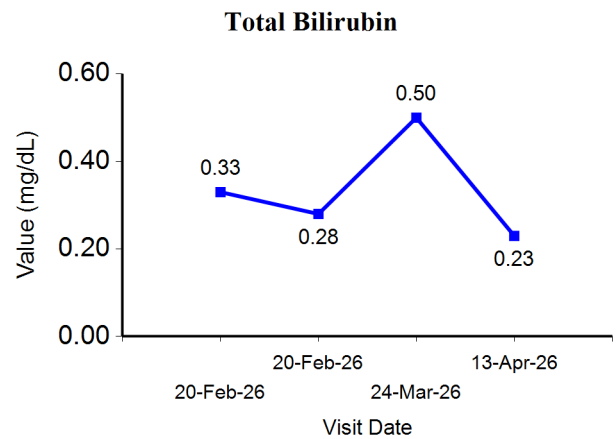
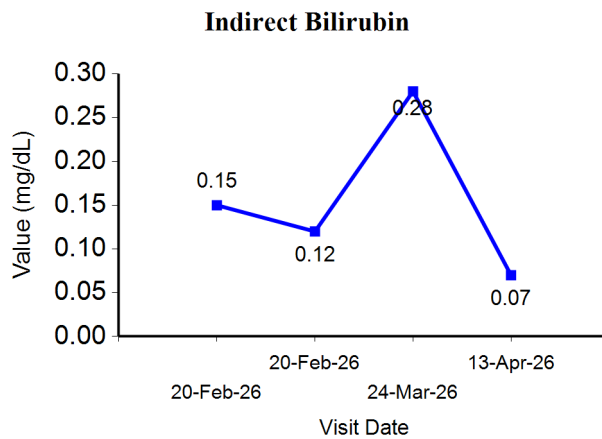


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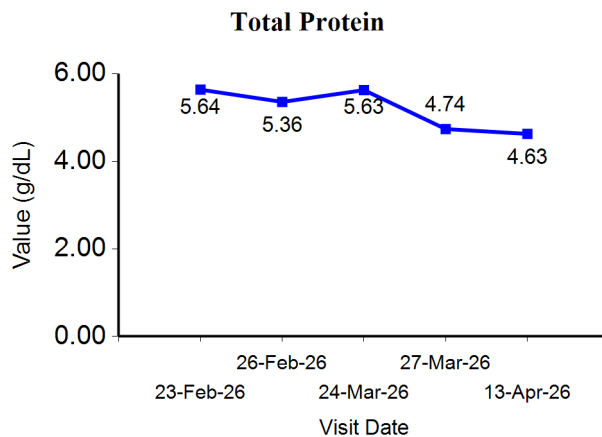
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∞ End of Report ∞

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